

Intake Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato

Supervisor: Erica Johnson

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 6/11/2025 2:30 PM - 3:52 PM

Duration: 82 minutes

Service Code: 90791

Location: Main Office

Participants: Client only

Presenting Problem

Client reports "I struggle with anxiety, I am a pretty anxious person and struggle to work with my emotions when they come up and struggle to evaluate why I am having the emotions I am having." Client reports "due to all of the life events I've experienced recently, it has intensified and exacerbated the anxiety that has been there my whole life. Along with feeling pretty depressed lately as a result of the life events."

Current Mental Status

Orientation:	X3: Oriented to Person, Place, and Time
General Appearance:	Appropriate
Dress:	Appropriate
Motor Activity:	Unremarkable
Interview Behavior:	Appropriate
Speech:	Normal
Mood:	Anxious
Affect:	Congruent
Insight:	Excellent
Judgment/Impulse Control:	Excellent
Memory:	Intact
Attention/Concentration:	Good
Thought Process:	Unremarkable
Thought Content:	Appropriate
Perception:	Unremarkable
Functional Status:	Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Objective Content

Therapist went over informed consent, policies, mandatory reporting, billing and cancellations.

Biopsychosocial Assessment

Identification:	Client reports being a "17 year old white female who is not religious." She/ her pronouns.
History of Present Problem:	Client reports "experiencing anxiety my whole life." Client reports "experiencing periods of time with depression and have been noticeably depressed for the last two months." Client reports "there is a low level of anxiety consistently throughout the day and experiences bouts of depression at points either everyday or multiple times a week."
Psychiatric History:	Client reports "in middle school I was in therapy and was on medication for anxiety and depression during Covid."
Trauma History:	Client reports "people close to me passed away and I had to put my dog down at the same

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time." Client reports "I was in a relationship and I requested for my needs to be met and he was unwilling to meet my needs and I had to break up with him." Client reports "brothers friend died in a skiing accident and brother was there and it took a toll on mine and my families life." Client reports "uncle that lived next door passed away while I was in middle school and I was really close to him." Client reports "I was bullied in elementary school." Client reports "in middle school I dated someone in who weaponized self-harm."

Family Psychiatric History: Client reports "my mom, dad, brother, and sister have all taken medications for mental health issues at some point." Client reports "dad's sister has some form of diagnosis surrounding depression." Client reports "aunt and extended family struggle with addiction and alcoholism."

Medical Conditions & History: Client reports "split my chin open ice skating when I was little." Client reports "having a ganglion cyst in my right wrist."

Allergies: Client reports "I have season allergies and an intolerance to dairy."

Dental: Client reports "none."

Current Medications: Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Substance Use: Client reports "none."

Family History: Client reports "I have a brother and sister. My sister is 6 years older and my brother is 3 years older." Client reports "my bother and I are close but he doesn't live at home and our communication isn't consistent. My sister and I are really close and hangout all the time." Client reports "my mom and I are close. My moms childhood was very traumatic, and that affected how she operated with self and us, some of her behaviors have negatively impacted my life, and she is very supportive and there for me." Client reports "my mom confides in me and I can feel put in the middle, but she has gotten better as I've gotten older." Client reports "my dad traveled all the time when I was younger for work and we weren't very close due to how he relates to me. We have gotten closer as I've gotten older."

Social History: Client reports "my best friend Malia has been my best friend since I was little and we can have fun, be genuine, and authentic with each other." Client reports "I have a lot of friends from many different friend groups. I tend to have friends all over." Client reports "a lot of my friends were a year older and graduated." Client reports "I have a new friend that formed at the start of the school year and she will tell me what I need to hear." Client reports "I have a lot of support in my life." Client reports "community resources include extra curricular school activities."

Spiritual/Cultural Factors: Client reports "I relate to the universe as the higher power, can be skeptical, logical, and believes in karma. The energy you put out is the energy you get back."

Developmental History: Client reports "I was diagnosed late with Dyslexia and ADHD in 5th grade." Client reports "being twice exceptional and can memorize and recall words and numbers."

Educational/Vocational History: Client reports "I know how to play six instruments, play guitar, I participated in choir, participate in the musicals at school, am on the school swim team, I used to bake a lot, and have learned how to do nails." Client reports "I used to work at sweet cow and lifeguard, and now I work at Rocky Mountian Tap and Garden." Client reports "I just finished my junior year of HS."

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Legal History:

Client reports "none."

SNAP:

Strengths: emotionally intelligent, can pick up on other peoples emotions and very empathetic and feel deeply, show up for people and I'm big on being there for people

Other Important Information:

Growth edges: struggle with being emotionally self directed, let emotions build up and don't know or understand why or where my emotions are coming from, can lash out, be rude, or withdraw when emotionally dysregulated and it is hard to evaluate for myself why that is
Not pregnant or desirous of pregnancy in the next twelve (12) months. Client already has access to contraceptive/family planning.

Client did not disclose perpetration of sexual or physical assault.

Client did not indicate challenges in ADLs: eating, bathing, getting dressed, toileting, mobility, and continence.

Client reports no barriers to treatment regarding cultural or power dynamics within the therapeutic relationship.

Client was in person at office for intake session.

Plan

Client reports "I would like to better myself, figure myself out, and have support in doing so."

Therapist will create a treatment plan between this session and the next. At the beginning of the next session therapist will go over treatment plan with client.

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Niccole Fortunato, Associate, signed this note and declared this information to be accurate and complete on 6/15/2025 at 8:17 AM.

Intake Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato

Supervisor: Erica Johnson

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 6/11/2025 2:30 PM - 3:52 PM

Duration: 82 minutes

Service Code: 90791

Location: Main Office

Participants: Client only

Erica Johnson, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 6/17/2025 at 9:51 AM.

Boulder Emotional Wellness

Date and Time: 6/15/2025 8:17 AM

Clinician: Niccole Fortunato

Supervisor: Erica Johnson

Patient: Gillian Sherry, DOB 5/14/2008

Diagnosis

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Diagnostic Justification

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D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Presenting Problem

Client reports "I struggle with anxiety, I am a pretty anxious person and struggle to work with my emotions when they come up and struggle to evaluate why I am having the emotions I am having." Client reports "due to all of the life events I've experienced recently, it has intensified and exacerbated the anxiety that has been there my whole life. Along with feeling pretty depressed lately as a result of the life events."

Treatment Goal

Client would like to better herself, figure herself out, and have support in doing so. Along with increasing knowledge and practice of tools and resourcing for emotional regulation and dysregulation to help support client as she is adjusting to multiple traumatic life events that occurred recently in a two week period.

Objective 1

Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation and client report in session.

Estimated Completion: 6 months (12/15/2025)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- EMDR grounding/resourcing exercises.
- Experiential Somatic Exercise.
- Exploration of Coping Patterns.
- Exploration of Emotions.

Boulder Emotional Wellness

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Supervisor: Erica Johnson

Patient: Gillian Sherry, DOB 5/14/2008

-
- Exploration of Nervous System Activation.
 - Exploration of Relationship Patterns.
 - Grounding/ resourcing.
 - Guided Imagery.
 - Interactive Feedback.
 - Interpersonal Resolutions.
 - Intrapersonal Resolutions.
 - Mindfulness Training.
 - Nervous System Exercise.
 - Nervous System Mapping.
 - Open Ended Questioning.
 - Parts work.
 - Practice Nonviolent Communication.
 - Preventative Services.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 2

Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Estimated Completion: 6 months (12/15/2025)

Treatment Strategy / Intervention

- Cognitive Refocusing.
- Cognitive Reframing.
- EMDR grounding/resourcing exercises.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Experiential Somatic Exercise.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.

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Supervisor: Erica Johnson

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-
- Interactive Feedback.
 - Interpersonal Resolutions.
 - Intrapersonal Resolutions.
 - Mindfulness Training.
 - Nervous System Exercise.
 - Nervous System Mapping.
 - Open Ended Questioning.
 - Parts work.
 - Preventative Services.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 3

Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms as client report in session.

Estimated Completion: 6 months (12/15/2025)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- EMDR grounding/resourcing exercises.
- Experiential Somatic Exercise.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Interactive Feedback.
- Interpersonal Resolutions.

Boulder Emotional Wellness

Date and Time: 6/15/2025 8:17 AM

Clinician: Niccole Fortunato

Supervisor: Erica Johnson

Patient: Gillian Sherry, DOB 5/14/2008

-
- Intrapersonal Resolutions.
 - Mindfulness Training.
 - Nervous System Exercise.
 - Nervous System Mapping.
 - Open Ended Questioning.
 - Parts work.
 - Practice Nonviolent Communication.
 - Preventative Services.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 4

There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Estimated Completion: 6 months (12/15/2025)

Prescribed Frequency of Treatment

Weekly

I declare that these services are medically necessary and appropriate to the recipient's diagnosis and needs.

Niccole Fortunato, Associate, signed this note and declared this information to be accurate and complete on 6/16/2025 at 10:38 AM.

Erica Johnson, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 6/17/2025 at 9:52 AM.

Treatment Plan

Boulder Emotional Wellness

Date and Time: 6/15/2025 8:17 AM

Clinician: Nicole Fortunato

Supervisor: Erica Johnson

Patient: Gillian Sherry, DOB 5/14/2008

Signature

I acknowledge that I have participated in the development of the treatment plan and agree with the goals, objectives, and interventions.

Gillian Sherry

Gillian Sherry

6/30/2025

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 6/16/2025 10:30 AM - 11:32 AM

Duration: 62 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

Insight: Excellent

Judgment/Impulse Control: Excellent

General Appearance: Appropriate

Memory: Intact

Dress: Appropriate

Attention/Concentration: Good

Motor Activity: Unremarkable

Thought Process: Unremarkable

Interview Behavior: Appropriate

Thought Content: Appropriate

Speech: Normal

Perception: Unremarkable

Mood: Anxious

Functional Status: Intact

Affect: Congruent

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I get frustrated." Client reports "it's all consuming when I think she is mad at me." Client reports "I am trying to prevent them from being in a bad mood."

Objective Content

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Boulder Emotional Wellness

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Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

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Duration: 62 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Therapist and client started with logistical stuff including payment and going over client's TP. Client signed off on goals and objectives offered in TP. Therapist then held space for client to share where she wanted to go in therapy. Client expressed feeling "nervous and not knowing where to start." Therapist offered emotional validation and gave client a few directions we could go in. Along with asking client what her nervousness needed to feel more comfortable. Client shared what came up for her in therapist inquiry and then spoke to relational dynamics with her mom. Therapist continued to offer emotional validation, along with supportive reflections, psych-ed, and asked open ended questions. Client is able to articulate her experience and also struggles to know or understand where some of her coping mechanisms and strategies stem from.

Interventions Used

The following interventions were used: Interactive Feedback, Cognitive Reframing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Supportive Reflection, Review of Treatment Plan/Progress, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at the office for session.

Plan

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 6/16/2025 10:30 AM - 11:32 AM

Duration: 62 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Therapist and client are in the trust building stages of the therapeutic relationship. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, Associate, signed this note and declared this information to be accurate and complete on 6/16/2025 at 11:40 AM.

Erica Johnson, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 6/17/2025 at 9:53 AM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 6/26/2025 10:00 AM - 11:03 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I am quick to get negative with myself but not with others." Client reports "I struggle with vulnerability." Client reports "the self-aggression tendencies can be motivating."

Objective Content

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

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Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 6/26/2025 10:00 AM - 11:03 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Therapist held space for client to check in. Client shared how she was feeling and named feeling "nervous." Therapist inquired with client about her nervousness. Client shared why she was feeling nervous and how it is connected to the part of her that is self-aggressive. Therapist and client explored client's self-aggressive tendencies and behaviors. Therapist asked open ended questions, offered psych-ed, and emotional validation. Client spoke to the "discomfort" that comes from feeling "vulnerable or expressing emotions." Client's tendencies are to self-isolate, shut down/ push away, or avoid her feelings. Therapist is working with client to create space where client can start to build capacity for "vulnerability and discomfort." Therapist invited client to practice somatic exercise to see where her capacity to sit in her discomfort is. Client participated in exercise and stated "I went into my breath so I didn't have to be with the thoughts."

Interventions Used

The following interventions were used: Interactive Feedback, Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at the office for session.

Plan

Progress Note

Boulder Emotional Wellness

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Patient: Gillian Sherry, DOB 5/14/2008

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Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Therapist and client are in the trust building stages of the therapeutic relationship. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/17/2025 at 3:18 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/18/2025 at 2:56 PM.

Progress Note

Boulder Emotional Wellness

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Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

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Location: Main Office

Participants: Client Only

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D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

Insight: Excellent

Judgment/Impulse Control: Excellent

General Appearance: Appropriate

Memory: Intact

Dress: Appropriate

Attention/Concentration: Good

Motor Activity: Unremarkable

Thought Process: Unremarkable

Interview Behavior: Appropriate

Thought Content: Appropriate

Speech: Normal

Perception: Unremarkable

Mood: Anxious

Functional Status: Intact

Affect: Congruent

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I noticed when I am being more self-critical and it is happening everyday." Client reports "the way I was treated created a lot of resentment." Client report "conflict stresses me out."

Objective Content

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Duration: 61 minutes

Service Code: 90837

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Participants: Client Only

Therapist held space for client to check in. Client shared feeling "anxious" and shared beliefs she holds connected to the anxiety. As well as new awareness she is having connected to her "self-criticism." Therapist listened as client shared and inquired with client around how the new awareness is impacting her. Client shared the ways she worked with her self-criticism when it arose and stated "I didn't realize how often it came up." Therapist offered supportive reflections, emotional validation, and asked open ended questions. Client then spoke to the relationship dynamics within her family system and how she has been impacted by those dynamics. As well as how they are connected to her "self-critic." Therapist offered psych-ed, reframes, and used parts work with client's self-critic. Client engaged with interventions and participated in parts work exercises. Client also stated feeling "less anxious" by the end of session.

Interventions Used

The following interventions were used: Interactive Feedback, Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

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Progress: Progressing

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Progress: Progressing

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Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at the office for session.

Plan

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 6/30/2025 10:30 AM - 11:31 AM

Duration: 61 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Therapist and client are in the trust building stages of the therapeutic relationship. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/17/2025 at 3:18 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/18/2025 at 2:57 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 7/7/2025 10:30 AM - 11:33 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I'm just a worrier and I am worried about how other people are feeling." Client reports "I invalidate my own feelings and take other people's feelings very personal." Client reports "I am trying to avoid the shame that comes along with telling someone how I feel."

Objective Content

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 7/7/2025 10:30 AM - 11:33 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Therapist held space for client to check in. Client shared how she was feeling and how she was feeling about going to camp. Therapist listened as client shared. Client then spoke to the relationship dynamics within her extended family and nuclear family and the mental, physical, and emotional impacts of the dynamics. Therapist asked open ended questions, offered supportive reflections, and emotional validation. Client spoke to the internal patterns and negative self view that was created as a result of how she was related to emotionally. Therapist offered observations, psych-ed, reframes, and gave client space to practice what she would want to say to different family members if she felt safe doing so. Client participated in exercise and shared what came up for her and then shared how it felt to say that. Therapist and client are working on building client's capacity to express her needs and feelings to others.

Interventions Used

The following interventions were used: Interactive Feedback, Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Role-Play/Behavioral Rehearsal, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at the office for session.

Plan

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 7/7/2025 10:30 AM - 11:33 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Therapist and client are in the trust building stages of the therapeutic relationship. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/17/2025 at 3:18 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/18/2025 at 2:58 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 7/21/2025 10:30 AM - 11:30 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I was able to practice being more emotionally vulnerable while I was at camp." Client reports "consistency feels familiar, it feels uneasy to be in transition. It's unpredictable and I feel on edge." Client reports "if I'm stressed out, I shut down and withdraw" what she notices about her coping strategies when she is stressed out and emotionally overwhelmed.

Objective Content

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 7/21/2025 10:30 AM - 11:30 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Therapist held space for client to check in. Client shared how she was feeling and how her trip to camp was. As well as the relational dynamics that surfaced in her family system. Therapist listened as client shared, offering supportive reflections and asking open ended questions. Client then spoke to the shifts she has been noticing internally occur by implementing tools we have worked with in regards to her negative thoughts and stories. Therapist celebrated with client her progress and asked clarifying questions about client's family relational dynamics. Client responded to her experience of interacting with her family dynamics and spoke to her pattern of "shutting down" when she is "stressed." Therapist offered supportive reflections, emotional validation, and psych-ed. Client was open and receptive to therapist offerings and client expressed resonance with NS psych-ed offered. Therapist modeled and guided client through NS exercise to work with her stress. Client participated in exercise and stated feeling "more relaxed" and said "wow that works, I can feel the shift."

Interventions Used

The following interventions were used: Interactive Feedback, Cognitive Reframing, Structured Problem Solving, Relaxation/Deep Breathing, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Nervous System Exercise, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at the office for session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 7/21/2025 10:30 AM - 11:30 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Plan

Therapist and client are in the trust building stages of the therapeutic relationship. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/28/2025 at 1:20 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/29/2025 at 5:10 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 7/28/2025 10:43 AM - 11:47 AM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I benefit from structure and get sad and down when there is a lot of downtime. I am looking forward to going back to school and being in my structure again." Client reports "arguing and being angry feel unproductive" in regards to how she relates to her mom and sisters way of being with their sympathetic response patterns. Client reports "I don't take it personally, I have normalized it and am indifferent" in regards to how she relates to relationships where distance has been a big part of the dynamic.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 7/28/2025 10:43 AM - 11:47 AM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Objective Content

Therapist held space for client to check in. Client started by sharing how she was doing and then also spoke to getting prepared for school to start back up. Client shared what that looks like for her and how that is making her feel. Therapist listened as client shared. Therapist and client then explored client's family relationship dynamics. Client spoke to the dynamics between her and her parents and her siblings. Client shared the impact of watching her parents behavior and how she relates to herself and others in an emotional context as a result. Therapist asked open ended questions, offered supportive reflections, emotional validation, and psych-ed. Therapist and client explored attachment styles. Therapist provided a video on attachment styles. Client watched video and shared what she noticed come up for her as a result. This video led into a conversation about client's own aversion to emotional dysregulation. Client shared her beliefs and the "indifference" she experiences now. Therapist offered reframes, used parts work, and gave client HW to practice working with this new awareness around "indifference." Client expressed willingness to track her indifference and report back what she notices.

Interventions Used

The following interventions were used: Interactive Feedback, Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at the office for session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 7/28/2025 10:43 AM - 11:47 AM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Plan

Therapist and client are in the trust building stages of the therapeutic relationship. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 7/28/2025 at 4:37 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 7/29/2025 at 5:11 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 8/4/2025 10:31 AM - 11:34 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I ruminate or don't think and am just sad" what happens for client when she isn't productive or staying consistently active.

Client reports "it is a deeply ingrained pattern and it makes me second guess myself more" the impact of the dynamics with her parents.

Client reports "I was more angry and sad" the impact of reflecting on the relationship dynamics with herself connected to childhood dynamics with her parents.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 8/4/2025 10:31 AM - 11:34 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Objective Content

Therapist held space for client to check in. Client started by sharing how she was doing and then shared the prolonged impact of a realization she had in the previous session connected to client's relationship with her father. Therapist listened as client shared, offering supportive reflections and asking open ended questions. Therapist and client continued to explore client's relationship dynamics with her father and also worked with client's relationship dynamics with her mother. Client spoke to the impact the dynamics between her and her parents have on her and the connection between the dynamics and her self-critic. Therapist offered emotional validation, and reframes. Client is able to acknowledge the "contradicting messages I received were confusing" and have led to a deeply "ingrained pattern" in myself and how I relate to myself. Client struggles to offer herself positive reinforcement and receive compliments as a result. Therapist used parts work, offered psych-ed, and grounding and resourcing tools to work with her self-critic. Client engaged with interventions used.

Interventions Used

The following interventions were used: Interactive Feedback, Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at the office for session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 8/4/2025 10:31 AM - 11:34 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Plan

Therapist and client are in the trust building stages of the therapeutic relationship. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 8/11/2025 at 7:40 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 8/12/2025 at 4:53 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 8/12/2025 4:15 PM - 5:20 PM

Duration: 65 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I am nervous about going back to school. I'm nervous about being in the unknown" related to the decision she made in regards to school. Client reports feeling "nervous" about meeting new people and stated the fears connected to the nervousness is "not being liked, messing up, and being/ feeling rejected." Client reports "I put a lot of weight into the statements" the impact of the messages she received as a child.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 8/12/2025 4:15 PM - 5:20 PM

Duration: 65 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Objective Content

Therapist held space for client to check in. Client started by sharing how she is doing and spoke to the transition she is navigating around school starting. Client shared decisions she has made related to school and the impact of those choices. Therapist listened as client shared, offering supportive reflections and asking open ended questions. Client responded to questions. Therapist and client then explored client's relationship to her emotions, her relationship dynamics, and coping strategies and the interconnection between the three. Client spoke to how the way she works with her emotions is due to how she was treated and how she observed others emotional responses and how that is connected to her self-critic. Therapist offered emotional validation, reframes, psych-ed, and used parts work. Client expressed resonance with psych-ed offered and shared more about the impact of the messages she received as a child in relation her to her emotional experiences. Therapist offered HW for client to track her somatic response to her intrapersonal and interpersonal dynamics. Client expressed willingness to track her experience and report back what she notices.

Interventions Used

The following interventions were used: Interactive Feedback, Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 8/12/2025 4:15 PM - 5:20 PM

Duration: 65 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Client was in person at the office for session.

Plan

Therapist and client are in the trust building stages of the therapeutic relationship. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 8/12/2025 at 9:53 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 8/14/2025 at 2:14 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 8/27/2025 5:33 PM - 6:33 PM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I was stressed out and had a feeling I was going to get in trouble" what client noticed happen during the interaction with her mom. Client reports "she brought in dynamics we weren't even talking about. I was trying to share an experience with her and she made it about what I did wrong and called me cold" the client's experience of the interaction with her mother. Client reports "her automatic assumption was hurtful" the impact of her moms behavior.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 8/27/2025 5:33 PM - 6:33 PM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and an interaction she had with her mom. Therapist listened as client shared. Client stated feeling "confused" by the dynamics that showed up between her and her mom. Client shared the prolonged impact of the dynamics on her mentally, physically, and emotionally. Therapist offered emotional validation, supportive reflections, psychoeducation, and asked open-ended questions. Therapist and client explored the "shame" that arose for client within the dynamics. Client shared that her inner critic has been louder since the interaction she was discussing. Client has been trying to figure out her role in the interaction. Client asked for clarification around therapist perspective. Therapist asked the client questions about how client relates to herself and the interaction. Therapist is working with client to build her internal trust and discernment around relational interactions and dynamics. Therapist continued to offer psychoeducation, reframes and emotional validation. Client stated feeling "less confused" and noticing her inner critic softening by the end of session. Therapist gave client homework to practice working with her inner critic using parts work and grounding and resourcing tools and exercises. Client expressed willingness to practice homework offered. Client will report back what she notices in the next session.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 8/27/2025 5:33 PM - 6:33 PM

Duration: 60 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Assessment / Additional Notes

Client was at home for telehealth session.

Plan

Therapist and client plan to continue exploring client's anxiety and how it shows up for the client interpersonally and intrapersonally. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience. Therapist and client are continuing to track client's emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 8/30/2025 at 5:21 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 9/3/2025 at 4:06 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 9/3/2025 6:30 PM - 7:33 PM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports feeling "stressed. It feels like there is this very looming thing" client sharing how she feels. Client reports "I was super pissed off because I couldn't figure something out and I was made to feel like I did something wrong because I expressed anger" client speaking to the interaction with her mom. Client reports "it's sad, but I am indifferent because it's my reality" the emotional impact the clients mom's behavior has on her.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 9/3/2025 6:30 PM - 7:33 PM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and then shared an interaction she had with her mother. Therapist listened as client shared. Therapist and client explore the interaction. Client spoke to the mental, physical, and emotional impacts her mom's behavior has on her. Client states her mom's behavior is linked to her perfectionistic tendencies. Therapist offered supportive reflections, emotional validation, and asked open-ended questions. Client continued to talk about how it feels to not be able to express emotions without it being turned into something she did wrong. Therapist offered psychoeducation and reframes. Therapist and client also explored clients stress level. Client stated feeling "stressed and overwhelmed" by all all of the tasks she has to complete related to school, work, self, and family. Therapist offered grounding and resourcing tools for client to practice to work with managing her stress level. Client expressed understanding and willingness to practice and report back what she notices.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Client was in person at the office for session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 9/3/2025 6:30 PM - 7:33 PM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Plan

Therapist and client plan to continue exploring client's anxiety and how it shows up for the client interpersonally and intrapersonally. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience. Therapist and client are continuing to track client's emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 9/4/2025 at 8:14 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 9/5/2025 at 8:18 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 9/10/2025 6:30 PM - 7:32 PM

Duration: 62 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "I was feeling so stressed out and I told my mom and she said something unhelpful" client speaking to the dynamics with her mom and the impact of going to her mom for support. Client reports "I completely doubt myself when she asks those questions" client speaking to the impact of the way her mom approaches questions about her experience. Client reports "I felt off today because of the school shooting that occurred. I was feeling very deflated and very hopeless" client, speaking to the impact of being a high school student and the fear connected to the lack of gun control laws.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 9/10/2025 6:30 PM - 7:32 PM

Duration: 62 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Objective Content

Therapist help space for client to check in. Client shared how she was feeling and updates on stressors around school and interpersonal relationship dynamics with her mom. Therapist listened as client shared. Therapist and client first explored clients stressors around school. Client shared the overwhelm that she was experiencing, as well as, the pressures of applying for colleges, studying, participating in extracurricular activities, working and trying to implement self-care activities. Client also acknowledged the impact of a school shooting happening that day and the "fear" it creates internally as a result. Therapist offered emotional validation, supportive reflections, and asked open ended questions. Client then spoke to the relationship dynamics with her mom. Client shared the pain points connected to being name called and the experience of having the negatives pointed out. Therapist continued to offer emotional validation. Therapist also offered reframes, psychoeducation, used parts work, and offered grounding and resourcing tools and reminders. Client engaged with interventions used.

Interventions Used

The following interventions were used: Cognitive Reframing, Structured Problem Solving, Cognitive Refocusing, Exploration of Emotions, Cognitive Challenging, Exploration of Coping Patterns, Communication Skills, Exploration of Relationship Patterns, Mindfulness Training, Preventative Services, Supportive Reflection, Symptom Management, Interpersonal Resolutions, Psycho-Education, Parts work, Open Ended Questioning, Exploration of Nervous System Activation, Grounding/ resourcing, and Intrapersonal Resolutions.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 9/10/2025 6:30 PM - 7:32 PM

Duration: 62 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Assessment / Additional Notes

Client was in person at the office for therapy session.

Plan

Therapist and client plan to continue exploring client's anxiety and how it shows up for the client interpersonally and intrapersonally. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience. Therapist and client are continuing to track client's emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 9/12/2025 at 9:14 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 9/13/2025 at 9:34 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 9/17/2025 6:30 PM - 7:34 PM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

Insight: Excellent

Judgment/Impulse Control: Excellent

General Appearance: Appropriate

Memory: Intact

Dress: Appropriate

Attention/Concentration: Good

Motor Activity: Unremarkable

Thought Process: Unremarkable

Interview Behavior: Appropriate

Thought Content: Appropriate

Speech: Normal

Perception: Unremarkable

Mood: Euthymic

Functional Status: Intact

Affect: Congruent

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Client reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Client reports "it feels like we're being screwed for the decisions being made and we can't do anything about it" client speaking to the impact of the gun violence threat at her school. Client reports "I'm stressed and anxious and I have a lot going on" client speaking to the stressors connected to school and graduation. Client reports "I don't understand why she says those things, it's unprovoked" client speaking to the relationship dynamics with her mom.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 9/17/2025 6:30 PM - 7:34 PM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Objective Content

Therapist held space for client to check in. Client shared how she was feeling and shared that there was a gun violence threat at her school. Client shared the impact of the threat. Therapist listened as client shared, offering emotional validation and asking open-ended questions. Client then spoke to the stressors around school and graduation. Along with sharing relationship dynamics that occurred and continue to show up with her mom. Therapist and client explored the relationship dynamics with the clients mom. Client named the mental, physical, and emotional impacts her moms statements towards her, have on her. Client expressed feeling "confused and not understanding." Therapist offered supportive reflections, psych-ed, reframes, and asked open-ended questions. Therapist and client then moved into talking about the impact of client losing one of her friends in a car accident this year. Client acknowledged the impact losing her friend at such a young age is having on her mentally and emotionally. Client spoke to "the sadness and reminders of all of the things they were supposed to do together." Therapist gave client space to just share about her experience navigating the loss, and continued to offer emotional validation. Client engaged with interventions used.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 9/17/2025 6:30 PM - 7:34 PM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Client was in person at the office for therapy session.

Plan

Therapist and client plan to continue exploring client's anxiety and how it shows up for the client interpersonally and intrapersonally. Therapist will hold space for client to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build client's capacity to be with her emotional experience. Therapist and client are continuing to track client's emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 9/18/2025 at 8:10 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 9/18/2025 at 8:38 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 10/1/2025 6:32 PM - 7:31 PM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

Insight: Excellent

Judgment/Impulse Control: Excellent

General Appearance: Appropriate

Memory: Intact

Dress: Appropriate

Attention/Concentration: Good

Motor Activity: Unremarkable

Thought Process: Unremarkable

Interview Behavior: Appropriate

Thought Content: Appropriate

Speech: Normal

Perception: Unremarkable

Mood: Anxious

Functional Status: Intact

Affect: Congruent

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

clt. reports "I am stressed about my to do list and everything I have to do. I feel overwhelmed by it all" clt. speaking to her emotions.

clt. reports "I wasn't taught how to rest. Rest was criticized if you weren't consistently busy" client speaking to how her pattern of staying busy developed and how it was reinforced.

clt. reports "fighting with her mom and her moms' consistent criticism" affects how she views and relates to herself.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 10/1/2025 6:32 PM - 7:31 PM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Objective Content

th. and clt. explored client's emotions. clt. shared feeling "stressed" and spoke to the aspects in her life that are causing her to feel stressed. th. offered emotional validation, supportive reflections, and asked open ended questions. th. and clt. explored milestones clt. is hitting in relation to school and getting herself ready to transition from HS to college and studying abroad. clt. shared her pattern of "constantly staying busy" supports all of the tasks she has to manage but also causes her "stress." clt. shared noticing trying to write lists of her tasks she "struggles to stay consistent with." th. explored with client what leads to the struggle. clt. shared what she notices about her patterning "that doesn't allow for consistency to occur." th. offered psych-ed, grounding and resourcing tools, and reframes. th. and clt. explored clt's. relationship dynamics. clt. acknowledged her insecurities and the relationship dynamics that impact her self-view. clt. stated feeling "impacted by others' perceptions of myself. what others say is ingrained in." th. worked with clt. on ways to relate to herself differently and offered emotional validation and used Parts work to work with insecure parts. clt. engaged with interventions used and expressed appreciation for th. support.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Experiential Somatic Exercise, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 10/1/2025 6:32 PM - 7:31 PM

Duration: 59 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Assessment / Additional Notes

clt. was at home for telehealth session.

Plan

th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 10/5/2025 at 8:20 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 10/7/2025 at 3:43 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 10/8/2025 6:30 PM - 7:34 PM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

Insight: Excellent

Judgment/Impulse Control: Excellent

General Appearance: Appropriate

Memory: Intact

Dress: Appropriate

Attention/Concentration: Good

Motor Activity: Unremarkable

Thought Process: Unremarkable

Interview Behavior: Appropriate

Thought Content: Appropriate

Speech: Normal

Perception: Unremarkable

Mood: Anxious

Functional Status: Intact

Affect: Congruent

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

clt. reports "it feels more like an obligation that I created based on how I saw her respond to my brother and sister" clt. speaking to the coping strategies and self-criticism she experiences around the relationship dynamics with her mom.

clt. reports "I get sad, blame myself, and am grossly disappointed in myself" clt. sharing what happens mentally and emotionally when she doesn't reach a goal she sets for herself. clt. acknowledges her self-criticism comes from the criticism she witnessed occur between family members in her family system.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 10/8/2025 6:30 PM - 7:34 PM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

clt. reports "I have an avoidance of criticism" clt. sharing the motivation behind the actions she takes and how she shows up in relationship dynamics.

Objective Content

clt. started by sharing updates on how her school stressors are going. clt. then moved into sharing dynamics that occurred between her and her family members. th. listened as clt. shared. th. and clt. explored clt's. relationship dynamics with her family members, predominantly her mom. clt. shared the mental, physical, and emotional impacts of how her mom relates to her have had on her. clt. stated that her "self-criticism" is a result of the way she saw her mom engage with her older siblings and father. clt. also shared a restorative experience she had with her dad and the "sadness" she felt as a result. th. asked open ended questions, offered supportive reflections, emotional validation, reframes, and psych-ed. th. noticed clt. started to distract after she spoke about the dynamics with her family system. th. asked clt. what she noticed happen. clt. expressed feeling anxious. th. shared psych-ed on NS states of activation and invited clt. to practice grounding and resourcing tools to work with decreasing clt's. anxiety and regulating clt's. NS. clt. participated in exercises and shared what she noticed about how her system felt before, during, and after the exercises. clt. stated feeling "better and less anxious" afterwards and by the end of session.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Relaxation/Deep Breathing, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 10/8/2025 6:30 PM - 7:34 PM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in person at the office for therapy session.

Plan

th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 10/8/2025 at 7:56 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 10/8/2025 at 8:24 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 10/22/2025 6:30 PM - 7:32 PM

Duration: 62 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

Insight: Excellent

Judgment/Impulse Control: Excellent

General Appearance: Appropriate

Memory: Intact

Dress: Appropriate

Attention/Concentration: Good

Motor Activity: Unremarkable

Thought Process: Unremarkable

Interview Behavior: Appropriate

Thought Content: Appropriate

Speech: Normal

Perception: Unremarkable

Mood: Anxious

Functional Status: Intact

Affect: Congruent

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

clt. reports "it felt mean and intentional and I got quiet" clt. speaking to the impact of the interaction with her friend.

clt. reports "I feel frustrated and I hate feeling underappreciated" clt. sharing the emotional impact and the belief she holds in regards to her friends behavior.

clt. reports "she treats me like I always have malicious intent and I don't understand why that's how she always treats me" clt. speaking to the relationship dynamics with her mom.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 10/22/2025 6:30 PM - 7:32 PM

Duration: 62 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Objective Content

clt. started by sharing how she was feeling and an interaction that occurred in one of her close friendships. th. listened as clt. shared. th. and clt. explored clt's. relationship dynamics. clt. shared the dynamics that occurred between her and her friend. clt. spoke to the impact the dynamics had on her mentally and emotionally. th. asked open ended questions, offered supportive reflections, and emotional validation. th. and clt. also spoke to clt's. relationship dynamics with her mom. clt. shared an interaction that occurred and the impact the interaction had on her. clt. also spoke to the coping strategies that arose in each of the relationship dynamics with her friend and her mom. clt. shared "shutting down and going blank" when conflict is present. clt. also is able to acknowledge that another strategy of hers is to take on the blame and question if she did something wrong. th. offered psych-ed on PRI and the protectors that arise when we feel rejected. clt. resonated with feeling "rejected." th. also offered reframes, and continued to offer emotional validation, supportive reflections, and ask open ended questions. clt. engaged with interventions used throughout session.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 10/22/2025 6:30 PM - 7:32 PM

Duration: 62 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Assessment / Additional Notes

clt. was in person at the office for therapy session.

Plan

th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 10/22/2025 at 7:46 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 10/22/2025 at 7:48 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 11/18/2025 7:15 PM - 8:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

Insight: Excellent

Judgment/Impulse Control: Excellent

General Appearance: Appropriate

Memory: Intact

Dress: Appropriate

Attention/Concentration: Good

Motor Activity: Unremarkable

Thought Process: Unremarkable

Interview Behavior: Appropriate

Thought Content: Appropriate

Speech: Normal

Perception: Unremarkable

Mood: Anxious

Functional Status: Intact

Affect: Congruent

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

clt. reports "I am nervous and scared about the unknown" clt. sharing her fears connected to clt's. upcoming experience.

clt. reports "I don't like being out of control" speaking to exploration of coping mechanisms.

clt. reports "I didn't say anything, but I have created some distance which has felt good" clt. speaking to the impact of a rupture in one of her relationship dynamics with a friend.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 11/18/2025 7:15 PM - 8:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Objective Content

clt. started by sharing how she was feeling and updates on school stressors. th. listened as clt. shared. th. and clt. explored clt's. relationship to the school stressors she was speaking to. clt. shared feeling "mad, annoyed, and frustrated." clt. was also able to acknowledge "I did the best I could" in which she also stated "I would have said that in the past" meaning before therapy. th. asked open ended questions, offered supportive reflections, emotional validation, and celebrated clt's. progress with her. th. and clt. also explored an upcoming experience clt. shared how she was feeling regarding the experience and expressed having heightened "anxiety." clt. also spoke to relationship dynamics with her family, friends, and partner. clt. stated "I am learning to not take on so much responsibility in my relationships." th. continued to ask open ended questions, offer supportive reflections, and emotional validation. th. also offered reframes and observations. clt. engaged with interventions used throughout session.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in person at the office for therapy session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 11/18/2025 7:15 PM - 8:21 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Plan

th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 11/20/2025 at 9:27 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 11/21/2025 at 4:12 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 11/26/2025 8:31 AM - 9:31 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and
Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Excellent

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

clt. reports "she got mad at me and then she was being weird with me. I assume a lack of value and care in how she responds" clt. speaking to the relationship dynamics with her mom.

clt. reports "I'm always doing a lot to overcompensate. I feel responsible. It creates anxious thought loops" clt. continuing to share the emotional and mental impact the relationship dynamics with her mom have on her.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 11/26/2025 8:31 AM - 9:31 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

clt. reports "my moms opinion is so important to me. I seek validation from her and that's why it hurts so much when she's mean to me. I am sensitive to tones and shifts in voices" clt. speaking more about the relationship dynamics with her mom.

Objective Content

clt. started by sharing how she was feeling and spoke to the relationship dynamics between her and her mom. th. listened as clt. shared. clt. spoke to the impact the relationship dynamics with her mom have on her mentally, physically, and emotionally. clt. stated " I believe I'm always doing something wrong. I got super sad and cried a lot. I try to do everything right and it's still always my fault" in response to how her mom talks to her and treats her. th. asked open-ended questions, offered supportive reflections, and emotional validation. th. and clt. explored ways to work with clt's. belief systems and emotions. clt. continued to share the impact her relationship with her mom has on her. th. offered psych-ed, reframes, and grounding and resourcing tools for client to use to work with her emotions and belief systems. clt. was open and receptive to th. offerings and shared the ways she is practicing "not taking responsibility" even though that is her current initial response.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 11/26/2025 8:31 AM - 9:31 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Assessment / Additional Notes

clt. was in person at the office for therapy session.

clt. has consistent underlying anxiety that increases when clt's. NS is triggerd/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and to continue to building and applying grounding and resourcing tools.

Plan

th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 12/7/2025 at 8:17 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 12/9/2025 at 1:36 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 12/1/2025 4:46 PM - 5:50 PM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

Insight: Excellent

Judgment/Impulse Control: Excellent

General Appearance: Appropriate

Memory: Intact

Dress: Appropriate

Attention/Concentration: Good

Motor Activity: Unremarkable

Thought Process: Unremarkable

Interview Behavior: Appropriate

Thought Content: Appropriate

Speech: Normal

Perception: Unremarkable

Mood: Anxious

Functional Status: Intact

Affect: Congruent

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

clt. reports "I am trying to get better at going with the flow. I notice my moms way of responding and I don't want to respond like that" clt. speaking to how she is trying to work with the relationship dynamics and responses that was modeled to her.

clt. reports "I stepped into the mediator role in my family. I have to do it all of the time. I hate conflict it sets me off" clt. sharing how she mitigates conflict in her family system.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 12/1/2025 4:46 PM - 5:50 PM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

clt. reports "I'm still working on not taking responsibility. It's really hard not to do it and I'm not good at not doing it" clt. sharing the impact of the relationship dynamics within her family system and how her system has responded to those dynamics.

Objective Content

clt. started by sharing how she was feeling and spoke to the family dynamics in her nuclear family and the impact those dynamics have on her. th. listened as clt. shared. clt. spoke to the "anxiety and the hypervigilance" that she experiences as a result of the dynamics. clt. also acknowledged that she gets put into and takes on the role of being the mediator. clt. stated "I don't like sitting in the discomfort of being in conflict." th. offered supportive reflections, emotional validation, psych-ed, and asked open-ended questions. th. is tracking clt's. way of regulating her nervous system is to mitigate being in conflict by becoming the mediator/stepping into people pleasing behaviors. th. offered observations to the clt. The clt. agreed with th. observations and stated "I hate it. It sets me off" speaking to conflict. th. and clt. also explored clt's. emotions. clt. stated "there's always underlying anxiety." th. invited clt. to practice grounding and resourcing exercises to work with her anxiety. clt. participated in the exercises and stated "that was nice. It was relaxing." clt. also acknowledged her progress at the end of session and stated "I am feeling better and I am able to understand my emotions better and I feel less anxious cause I know tools of how to work with it." th. celebrated clt's. progress with her.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Guided Imagery, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Parts work, Preventative Services, Psycho-Education, Relaxation/Deep Breathing, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 12/1/2025 4:46 PM - 5:50 PM

Duration: 64 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in person at the office for therapy session.

clt. has "consistent underlying anxiety" that increases when clt's. NS is triggerd/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and continuing to build and apply clt's. grounding and resourcing tools.

Plan

th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 12/7/2025 at 9:50 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 12/9/2025 at 1:44 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 12/10/2025 6:30 PM - 7:36 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F43.23 Adjustment Disorder, With mixed anxiety and depressed mood

Diagnostic Justification

A. The development of emotional or behavioral symptoms in response to an identifiable stressor(s) occurring within 3 months of the onset of the stressor(s).

B. These symptoms or behaviors are clinically significant, as evidenced by one or both of the following:

Marked distress that is out of proportion to the severity or intensity of the stressor, taking into account the external context and the cultural factors that might influence symptom severity and presentation.

Significant impairment in social, occupational, or other important areas of functioning.

C. The stress-related disturbance does not meet the criteria for another mental disorder and is not merely an exacerbation of a preexisting mental disorder.

D. The symptoms do not represent normal bereavement.

E. Once the stressor or its consequences have terminated, the symptoms do not persist for more than an additional 6 months.

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

Insight: Good

Judgment/Impulse Control: Excellent

General Appearance: Appropriate

Memory: Intact

Dress: Appropriate

Attention/Concentration: Good

Motor Activity: Unremarkable

Thought Process: Unremarkable

Interview Behavior: Appropriate

Thought Content: Appropriate

Speech: Normal

Perception: Unremarkable

Mood: Anxious

Functional Status: Intact

Affect: Congruent

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

clt. reports "I am not the best with transitions they make me nervous" clt. noticing her anxiety is heightened as she is moving through multiple transitions occurring simultaneously.

clt. reports "I relate to life and death differently and it created a redirection. It changed how I think about a lot of things. It mentally changed me" clt. sharing about the impact her friends death has and continues to have on her.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 12/10/2025 6:30 PM - 7:36 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

clt. reports "I also had to mature faster and I had already had to mature faster since I felt responsible for others emotions from such a young age. I get so crippling anxious thinking about consequences or feeling so responsible for others feelings" clt. sharing an aspect of her experience and life that heightens and exacerbates her anxiety. clt. was reminded of this through the process of her friends passing and the loss of not being able to just be a kid.

Objective Content

th. and clt. started with reevaluating and updating clt's. TP. th. changed clt's. diagnosis from Adjustment Disorder, With mixed anxiety and depressed mood to Generalized Anxiety Disorder. clt's. anxiety is occurring persistently and consistently outside of the adjustment disorder specifications. th. updated clt's. presenting problem, goal, and objectives. clt. consented to goal and objectives created over the next six months. After th. and clt. finished clt's. TP, th. held space for clt. to share her experience. clt. spoke to the emotional impacts of her friend who passed this last years birthday anniversary. clt. stated "the whole day was a difficult day. I cried a lot and I tired pushing if off the next day but it was still there." th. and clt. explored the continued impact of her friends death and how transitions and life milestones reactivate the emotions and thoughts connected to her friends death. As well as, the loss of childhood connected to being anxious and feeling responsible for her moms and others emotions. th. offered supportive reflections, emotional validation, psych-ed, reframes, and asked open ended questions. clt. engaged with interventions used and was able to see progress milestones around addressing the emotional impact of her friends loss and the loss of childhood.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Psycho-Education, Review of Treatment Plan/Progress, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and implement self-soothing techniques to use at least 3x a week in order to decrease "anxiety and depression" while increasing emotional regulation aeb client report in session.

Progress: Progressing

2. Client will learn somatic based exercises in session to process through past traumas held and stored in the body to decrease "anxiety and depression" symptoms and learn tools to bring safety back into her body aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection in moments when she notices a wound is activated by tracking what happened before, during, or after to bring more awareness to and learn the patterns at play and how her body responds to track connections between activation and "anxiety and depression" symptoms aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 12/10/2025 6:30 PM - 7:36 PM

Duration: 66 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
4. There are currently no obstacles to treatment regarding cultural or power dynamics in therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

clt. was in person at the office for therapy session.

clt. has "consistent underlying anxiety" that increases when clt's. NS is triggered/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and continuing to build and apply clt's. grounding and resourcing tools.

Plan

th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 12/10/2025 at 8:18 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 12/11/2025 at 6:05 PM.

Boulder Emotional Wellness

Date and Time: 12/10/2025 6:32 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Presenting Problem

Client reports "I struggle with anxiety, I am a pretty anxious person and struggle to work with my emotions when they come up and struggle to evaluate why I am having the emotions I am having." Client reports "due to all of the life events I've experienced within the last year, it has intensified and exacerbated the anxiety that has been there my whole life." Client reports "anxiety is consistent and persistent daily, some days its more apparent than others." Client reports experiencing "heightened levels of anxiety occurring either all day or moments throughout the day. The intensity and frequency varies daily."

Treatment Goal

Client would like to better herself, figure herself out, and have support in doing so. Along with increasing knowledge and application of tools and resourcing for emotional regulation and dysregulation to decrease the intensity, frequency, and consistency of client's anxiety.

Objective 1

Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease the intensity, frequency, and consistency of client's "anxiety" and increase NS regulation. Client will decrease the distress

Boulder Emotional Wellness

Date and Time: 12/10/2025 6:32 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

connected to her "anxiety" levels from a level 8 to a level 6, over the next six months aeb client report in session.

Estimated Completion: 6 months (6/10/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.
- Communication Skills.
- EMDR grounding/resourcing exercises.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Interactive Feedback.
- Interpersonal Resolutions.
- Intrapersonal Resolutions.
- Mindfulness Training.
- Nervous System Exercise.
- Nervous System Mapping.
- Open Ended Questioning.
- Parts work.
- Practice Nonviolent Communication.
- Preventative Services.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Role-Play/Behavioral Rehearsal.
- Structured Problem Solving.
- Supportive Reflection.
- Symptom Management.

Objective 2

Client will learn and implement somatic based exercises in session to decrease the somatic, emotional and mental responses connected to the client's "anxiety." Client will decrease the emotional, physical, and mental responses from a level 8 to a level 6, over the next six months aeb client report in session.

Estimated Completion: 6 months (6/10/2026)

Boulder Emotional Wellness

Date and Time: 12/10/2025 6:32 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Treatment Strategy / Intervention

- Cognitive Refocusing.
- Cognitive Reframing.
- EMDR grounding/resourcing exercises.
- Exploration of Coping Patterns.
- Exploration of Emotions.
- Exploration of Nervous System Activation.
- Exploration of Relationship Patterns.
- Grounding/ resourcing.
- Guided Imagery.
- Interactive Feedback.
- Interpersonal Resolutions.
- Intrapersonal Resolutions.
- Mindfulness Training.
- Open Ended Questioning.
- Parts work.
- Preventative Services.
- Psycho-Education.
- Relaxation/Deep Breathing.
- Role-Play/Behavioral Rehearsal.
- Structured Problem Solving.
- Supportive Reflection.
- Symptom Management.
- Somatic Exercises.
- Validation Exercises.

Objective 3

Client will engage in active self-reflection and journaling at least 2x a week in moments after her NS has become activated and she is down regulated enough to track what happened before, during, or after NS response to bring more awareness to and learn the patterns at play and how her body responds to track connections between external and/ or internal stimulus and her NS response to decrease client's NS response time and activation period aeb client report in session.

Estimated Completion: 6 months (6/10/2026)

Treatment Strategy / Intervention

- Cognitive Challenging.
- Cognitive Refocusing.
- Cognitive Reframing.

Boulder Emotional Wellness

Date and Time: 12/10/2025 6:32 PM

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

-
- Communication Skills.
 - EMDR grounding/resourcing exercises.
 - Exploration of Coping Patterns.
 - Exploration of Emotions.
 - Exploration of Nervous System Activation.
 - Exploration of Relationship Patterns.
 - Grounding/ resourcing.
 - Guided Imagery.
 - Interactive Feedback.
 - Interpersonal Resolutions.
 - Intrapersonal Resolutions.
 - Mindfulness Training.
 - Nervous System Exercise.
 - Nervous System Mapping.
 - Open Ended Questioning.
 - Parts work.
 - Practice Nonviolent Communication.
 - Preventative Services.
 - Psycho-Education.
 - Relaxation/Deep Breathing.
 - Role-Play/Behavioral Rehearsal.
 - Structured Problem Solving.
 - Supportive Reflection.
 - Symptom Management.

Objective 4

There are currently no obstacles to treatment regarding cultural, power, or other dynamics in the therapeutic relationship aeb client report in session.

Estimated Completion: 6 months (6/10/2026)

Additional Information

Client reports noticing a decrease in her depression symptoms however, client noted she still experiences high levels of "anxiety" when her NS is activated and has consistent and persistent anxiety daily; continued therapy is needed to help client continue to work towards her goal of lowering her "anxiety" and NS response and increase her knowledge and application of tools and resources.

Therapist changed client's diagnosis from Adjustment Disorder, with Mixed Anxiety and Depressed Mood, to Generalized Anxiety Disorder due to client's symptoms of depression decreasing and client's symptoms of anxiety being consistent and persistent even

Treatment Plan

Boulder Emotional Wellness

Date and Time: 12/10/2025 6:32 PM

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

after the initial 6 month mark from the start of therapy.

Prescribed Frequency of Treatment

Weekly

I declare that these services are medically necessary and appropriate to the recipient's diagnosis and needs.

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 12/10/2025 at 8:25 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 12/11/2025 at 6:06 PM.

Signature

I acknowledge that I have participated in the development of the treatment plan and agree with the goals, objectives, and interventions.



Gillian Sherry
1/21/2026

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 12/19/2025 10:30 AM - 11:37 AM

Duration: 67 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Excellent

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 12/19/2025 10:30 AM - 11:37 AM

Duration: 67 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Clt. reports "I struggle with the not knowing and I haven't done anything big on my own so I don't know what to expect" clt. sharing what is leading to her anxiety.

Clt. reports "I am working on listening to my body's no" clt. share ways she is currently trying to alleviate some of her anxiety connected to upcoming decisions.

Clt. reports "I don't know what to do with myself if I am not learning or challenging myself academically" clt. speaking to the impact of identifying with learning and challenging self academically that is causing her distress.

Objective Content

Th. and clt. explored clt's. anxiety including stressors that increase her anxiety symptoms of "muscle tension, excessive worry, and hypervigilance leading to her feeling more restless." Clt. spoke to school stressors and upcoming experiences and decisions that are increasing her anxiety symptoms. Clt. stated "I identify with learning." Clt. graduated early and named realizing she is feeling anxious around how to fill her time. Clt. also stated feeling "nervous and anxious" about studying abroad by herself. Clt. has excessive worry with down time and transitions. Th. offered supportive reflections, emotional validation, and asked open ended questions. Th. and clt. explored resources clt. could practice to work with her anxiety and excessive worry. Th. provided resources for clt. to practice. Clt. was open and receptive to th. offerings and expressed willingness to practice resources offered.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Relaxation/Deep Breathing, Structured Problem Solving, Supportive Reflection, Symptom Management, and Validation Exercises.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease the intensity, frequency, and consistency of client's "anxiety" and increase NS regulation. Client will decrease the distress connected to her "anxiety" levels from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 12/19/2025 10:30 AM - 11:37 AM

Duration: 67 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
2. Client will learn and implement somatic based exercises in session to decrease the somatic, emotional and mental responses connected to the client's "anxiety." Client will decrease the emotional, physical, and mental responses from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection and journaling at least 2x a week in moments after her NS has become activated and she is down regulated enough to track what happened before, during, or after NS response to bring more awareness to and learn the patterns at play and how her body responds to track connections between external and/ or internal stimulus and her NS response to decrease client's NS response time and activation period aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in the therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was in person at the office for therapy session.

Clt. has "consistent underlying anxiety" that increases when clt's. NS is triggered/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and continuing to build and apply clt's. grounding and resourcing tools.

Trauma-focused treatment protocols required additional time to complete the interventions without destabilizing the client.

Plan

Th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. Th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. Th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 12/26/2025 at 4:26 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 12/28/2025 at 6:34 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/6/2026 10:00 AM - 11:03 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/6/2026 10:00 AM - 11:03 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Clt. is building her capacity to be emotional expressive and vulnerable. Clt. is usually emotionally reserved in session even in spite of expressing living in constant anxiety. Clt's. anxiety presented as sadness in session once she hit her threshold for the "stress, anxiety, and overwhelm."

Clt. reports "it's freaking me out. It makes me nervous" clt. speaking to the emotional impact her school stressors are having on her.

Clt. reports "it was consuming my thoughts and I woke up on the hour every hour" clt. speaking to negative impacts her anxiety connected to her school stressors are having on her.

Objective Content

Th. and clt. explored clt's. anxiety regarding school stressors and relationship dynamics that increase her anxiety symptoms of muscle tension, excessive worry, and hypervigilance. Clt. started to cry when she learned of a new information regarding her school stressors and the relationship dynamics connected to the school stressors. Clt. stated "this could have been prevented and all of my work didn't matter."

Th. offered supportive reflections, emotional validation, held space for clt. to emote, and asked open ended questions. Th. and clt. explored clt's. emotions. Clt. expressed having "constant anxious ruminating thoughts" about her school stressors. Th. continued to offer supportive reflections, emotional validation, and ask open ended questions. Along with offering reframes and psych-ed. Clt. was open and receptive to th. offerings and expressed feeling "better" by the end of session.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Relaxation/Deep Breathing, Structured Problem Solving, Supportive Reflection, Symptom Management, and Validation Exercises.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease the intensity, frequency, and consistency of client's "anxiety" and increase NS regulation. Client will decrease the distress connected to her "anxiety" levels from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/6/2026 10:00 AM - 11:03 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

2. Client will learn and implement somatic based exercises in session to decrease the somatic, emotional and mental responses connected to the client's "anxiety." Client will decrease the emotional, physical, and mental responses from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection and journaling at least 2x a week in moments after her NS has become activated and she is down regulated enough to track what happened before, during, or after NS response to bring more awareness to and learn the patterns at play and how her body responds to track connections between external and/ or internal stimulus and her NS response to decrease client's NS response time and activation period aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in the therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was in person at the office for therapy session.

Clt. has "consistent underlying anxiety" that increases when clt's. NS is triggered/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and continuing to build and apply clt's. grounding and resourcing tools.

Trauma-focused treatment protocols required additional time to complete the interventions without destabilizing the clt.

Plan

Th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. Th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. Th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 1/10/2026 at 6:24 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 1/12/2026 at 6:39 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/13/2026 10:00 AM - 11:00 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/13/2026 10:00 AM - 11:00 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Clt. reports "there is a lot of negative self-talk and the intrusive thoughts are constantly going on in the background."

Clt. reports "I am afraid of failure" clt. sharing her fears connected to her intrusive thoughts.

Clt. reports "I have a very active brain and my brain keeps saying to me 'what are you doing?'" clt. sharing one of the intrusive thoughts she is having connected to her system thinking/ believing she is not doing enough.

Objective Content

Th. and clt. explored clt's anxiety. Clt. shared having intrusive thoughts connected to her anxiety. Clt. stated "I keep having thoughts that I'm not doing enough." Th. listened as clt. shared offering supportive reflections and asking open ended questions. Th. and clt. explored clt's intrusive thoughts. Clt. expressed struggling to adjust to the readjustment period of "overdoing" when she was in school to having more down time. Th. offered supportive reflections, emotional validation, reframes, psych-ed, and grounding and resourcing tools for clt. to use to decrease her negative self-talk and intrusive thoughts. Clt. was open and receptive to th. offerings and expressed willingness to practice tools and resources offered and stated feeling "better" by the end of session.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease the intensity, frequency, and consistency of client's "anxiety" and increase NS regulation. Client will decrease the distress connected to her "anxiety" levels from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

2. Client will learn and implement somatic based exercises in session to decrease the somatic, emotional and mental responses connected to the client's "anxiety." Client will decrease the emotional, physical, and mental responses from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/13/2026 10:00 AM - 11:00 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
3. Client will engage in active self-reflection and journaling at least 2x a week in moments after her NS has become activated and she is down regulated enough to track what happened before, during, or after NS response to bring more awareness to and learn the patterns at play and how her body responds to track connections between external and/ or internal stimulus and her NS response to decrease client's NS response time and activation period aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in the therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was in person at the office for therapy session.

Clt. has "consistent underlying anxiety" that increases when clt's. NS is triggered/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and continuing to build and apply clt's. grounding and resourcing tools.

Plan

Th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. Th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience. Th. and clt. are continuing to track clt's. emotional distress levels, to decrease her anxiety interpersonally and intrapersonally.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 1/13/2026 at 11:15 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 1/14/2026 at 12:52 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/20/2026 10:01 AM - 11:04 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/20/2026 10:01 AM - 11:04 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Clt. reports "the fear comes from past dynamics and lives in the back of my head" speaking to her fears around "caring too much and being in the vulnerable position."

Clt. reports "it's hard to feel left behind" clt. sharing a belief she holds connected to her sadness.

Clt. reports "I am not good at feeling my feelings. I am very logical and realistic" clt. expressed struggling to be with her sadness and anxiety without going into story or having intrusive thoughts arise.

Objective Content

Th. and clt. explored clt's anxiety. Clt. shared relationship dynamics that are increasing her anxiety. Clt. stated "I am worried and scared and it feels like he doesn't care." Th. listened as clt. shared offering supportive reflections, emotional validation, and asking open ended questions. Th. and clt. explored clt's emotions. Clt. expressed struggling to not let her intrusive thoughts of "he doesn't care" connected to feeling vulnerable, sad, and scared arise. Th. offered observations, emotional validation, reframes, psych-ed, and grounding and resourcing tools for clt. to use to decrease her anxiety and intrusive thoughts. Clt. was open and receptive to th. offerings and expressed willingness to practice tools and resources offered and expressed feeling "better" by the end of session. Clt. also stated "it's helpful to understand why I was feeling the way I was feeling."

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, IFS, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Relaxation/Deep Breathing, Somatic Exercises, Structured Problem Solving, Supportive Reflection, Symptom Management, and Validation Exercises.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease the intensity, frequency, and consistency of client's "anxiety" and increase NS regulation. Client will decrease the distress connected to her "anxiety" levels from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

2. Client will learn and implement somatic based exercises in session to decrease the somatic, emotional and mental responses connected to the client's "anxiety." Client will decrease the emotional, physical, and mental responses from a level 8 to a level 6, over

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/20/2026 10:01 AM - 11:04 AM

Duration: 63 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

the next six months aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection and journaling at least 2x a week in moments after her NS has become activated and she is down regulated enough to track what happened before, during, or after NS response to bring more awareness to and learn the patterns at play and how her body responds to track connections between external and/ or internal stimulus and her NS response to decrease client's NS response time and activation period aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in the therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was in person at the office for therapy session.

Clt. has "consistent underlying anxiety" that increases when clt's. NS is triggered/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and continuing to build and apply clt's. grounding and resourcing tools.

Trauma-focused treatment protocols required additional time to complete the interventions without destabilizing the clt.

Plan

Th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. Th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience and decrease her anxiety concurrently.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 1/24/2026 at 1:39 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 1/26/2026 at 8:03 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/27/2026 11:00 AM - 12:00 PM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/27/2026 11:00 AM - 12:00 PM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clit. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Clit. reports "it was helpful to talk, have feelings, and not direct them at the person" clit. sharing about the impact of the resources offered in the last session. Th. and clit. made connection to the resource offered in the last session and clit's. anxiety connected to others heightened emotional states.

Clit. reports "making a mistake becomes crippling and I am a perfectionist" clit. sharing how her anxiety presents itself.

Clit. reports "I am getting better at setting boundaries and it's not as uncomfortable" clit. sharing other progress she is making in regards to areas that used to increase her anxiety.

Objective Content

Th. and clit. explored clit's. anxiety. Clit. shared another area she notices her anxiety becomes heightened. Clit. stated "it feels so real" regarding her anxious thoughts and feelings. Th. listened as clit. shared offering supportive reflections and emotional validation. Clit. then shared noticing a shift in her anxiety. Clit. stated "I am more relaxed and not as anxious all the time." Th. celebrated clit's. progress with her and explored what she notices have allowed these shifts to occur. Clit. shared what she thinks has lead to feeling more regulated in her experience and stated "I am less reactionary, not taking on others dysregulation, and I am able to think about it more." Clit. then moved into sharing about how other's heightened emotions increase her anxiety. Clit. shared what causes her anxiety to heighten. Clit. stated "there is a fear it will become my fault." Th. asked open ended questions, offered psych-ed, reframes, and grounding and resourcing tools for clit. to implement to work with decreasing her anxiety.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, IFS, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease the intensity, frequency, and consistency of client's "anxiety" and increase NS regulation. Client will decrease the distress connected to her "anxiety" levels from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 1/27/2026 11:00 AM - 12:00 PM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

-
2. Client will learn and implement somatic based exercises in session to decrease the somatic, emotional and mental responses connected to the client's "anxiety." Client will decrease the emotional, physical, and mental responses from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection and journaling at least 2x a week in moments after her NS has become activated and she is down regulated enough to track what happened before, during, or after NS response to bring more awareness to and learn the patterns at play and how her body responds to track connections between external and/ or internal stimulus and her NS response to decrease client's NS response time and activation period aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in the therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was in person at the office for therapy session.

Clt. has "consistent underlying anxiety" that increases when clt's. NS is triggered/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and continuing to build and apply clt's. grounding and resourcing tools.

Plan

Th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. Th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience and decrease her anxiety concurrently.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 2/1/2026 at 8:13 PM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 2/2/2026 at 8:30 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 2/3/2026 10:00 AM - 11:00 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Depressed

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 2/3/2026 10:00 AM - 11:00 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Clt. reports "I've been thinking about my friend and it has been on my mind constantly."

Clt. reports "I am feeling isolated and I was hurt by what one of my friends said and I don't understand why she would say what she said, it doesn't feel productive" clt. sharing more about what was causing her to feel down.

Clt. reports "I hadn't fully acknowledged how I was feeling and it feels good to acknowledge it" clt. speaking to the relationship dynamics in her friendships.

Objective Content

Clt. shared her anxiety was less present this past week and stated feeling "down" over the last few days. Clt. shared she has been having thoughts daily regarding her friends passing. Th. listened as clt. shared, asking open ended questions and offering supportive reflections. Th. and clt. explored why clt. has been having an increase in thoughts and emotions connected to her friends passing. Clt. acknowledged her friends death "doesn't feel fair" and she has been reminiscing about their friendship and feeling sadness at her friend who will not get to experience future life moments. Clt. shared she has also been experiencing loss and isolation in other friendships, which is leading to her heightened sadness and grief connected to her friends death. Th. provided emotional validation, reframes, and continued to ask open ended questions and offer supportive reflections. Clt. engaged with interventions used.

Interventions Used

The following interventions were used: Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, and Symptom Management.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease the intensity, frequency, and consistency of client's "anxiety" and increase NS regulation. Client will decrease the distress connected to her "anxiety" levels from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

2. Client will learn and implement somatic based exercises in session to decrease the somatic, emotional and mental responses connected to the client's "anxiety." Client will decrease the emotional, physical, and mental responses from a level 8 to a level 6, over the next six months aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 2/3/2026 10:00 AM - 11:00 AM

Duration: 60 minutes

Service Code: 90837

Location: Main Office

Participants: Client Only

Progress: Progressing

3. Client will engage in active self-reflection and journaling at least 2x a week in moments after her NS has become activated and she is down regulated enough to track what happened before, during, or after NS response to bring more awareness to and learn the patterns at play and how her body responds to track connections between external and/ or internal stimulus and her NS response to decrease client's NS response time and activation period aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in the therapeutic relationship aeb client report in session.

Progress: Progressing

Assessment / Additional Notes

Clt. was in person at the office for therapy session.

Clt. has "consistent underlying anxiety" that increases when clt's. NS is triggered/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and continuing to build and apply clt's. grounding and resourcing tools.

Plan

Th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. Th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience and decrease her anxiety concurrently.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 2/3/2026 at 11:14 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 2/3/2026 at 2:49 PM.

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 2/10/2026 10:04 AM - 11:02 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Diagnosis

F41.1 Generalized Anxiety Disorder

Diagnostic Justification

A. Excessive anxiety and worry (apprehensive expectation), occurring more days than not for at least 6 months, about a number of events or activities (such as work or school performance).

B. The individual finds it difficult to control the worry.

C. The anxiety and worry are associated with three (or more) of the following six symptoms (with at least some symptoms having been present for more days than not for the past 6 months);

Note: Only one item is required in children.

1. Restlessness or feeling keyed up or on edge.

2. Being easily fatigued.

3. Difficulty concentrating or mind going blank.

4. Irritability.

5. Muscle tension.

6. Sleep disturbance (difficulty falling or staying asleep, or restless, unsatisfying sleep).

D. The anxiety, worry, or physical symptoms cause clinically significant distress or impairment in social, occupational, or other important areas of functioning.

E. The disturbance is not attributable to the physiological effects of a substance (e.g., a drug of abuse, a medication) or another medical condition (e.g., hyperthyroidism).

F. The disturbance is not better explained by another mental disorder (e.g., anxiety or worry about having panic attacks in panic disorder, negative evaluation in social anxiety disorder [social phobia], contamination or other obsessions in obsessive-compulsive disorder, separation from attachment figures in separation anxiety disorder, reminders of traumatic events in posttraumatic stress disorder, gaining weight in anorexia nervosa, physical complaints in somatic symptom disorder, perceived appearance flaws in body dysmorphic disorder, having a serious illness in illness anxiety disorder, or the content of delusional beliefs in schizophrenia or delusional disorder).

Current Mental Status

Orientation: X3: Oriented to Person, Place, and Time

General Appearance: Appropriate

Dress: Appropriate

Motor Activity: Unremarkable

Interview Behavior: Appropriate

Speech: Normal

Mood: Anxious

Affect: Congruent

Insight: Good

Judgment/Impulse Control: Good

Memory: Intact

Attention/Concentration: Good

Thought Process: Unremarkable

Thought Content: Appropriate

Perception: Unremarkable

Functional Status: Intact

Progress Note

Boulder Emotional Wellness

Clinician: Niccole Fortunato, LPC

Supervisor: Erica Johnson, LPC

Patient: Gillian Sherry, DOB 5/14/2008

Date and Time: 2/10/2026 10:04 AM - 11:02 AM

Duration: 58 minutes

Service Code: 90837

Location: Teletherapy- Patient is Home

Participants: Client Only

Risk Assessment

Patient denies all areas of risk. No contrary clinical indications present.

Medications

Clt. reports "I have a hormonal IUD and take Spiral Naeliacto for acne."

Subjective Report and Symptom Description

Clt. reports "I set a boundary and the person didn't like they seemed annoyed with me afterwards" which led clt. to believe she had done something wrong.

Clt. reports "I wasn't mad, I was sad."

Clt. reports "I was crying because I felt left out" clt. building awareness and capacity for what is at the root of her emotional sadness connected to rejection.

Clt's anxiety becomes heightened when both of these beliefs the clt. has of believing "she did something wrong or feeling left out" come online.

Objective Content

Clt. shared an interpersonal interaction she had that activated her into heightened emotional states including "anxiety and sadness." Th. listened as clt. shared, tracking themes and patterns. Th. and clt. explored the self-aggressive and rejection beliefs and stories that arose for the clt. within the interaction. Clt. stated "I felt left out and stated my boundary and then felt like I did something wrong." Th. offered supportive reflections, emotional validation, reframes, psych-ed, and asked open ended questions. Th. and clt. explored how to resource clt. while she is in heightened emotional states and negative/ self-aggressive stories and beliefs come online. Th. provided grounding, resourcing, and validation exercises and tools for clt. to practice and implement to decrease her distress and increase her capacity for discomfort concurrently. Clt. practiced active listening and expressed understanding and resonance for information and resources th. provided and stated feeling "better" by the end of session.

Interventions Used

The following interventions were used: Cognitive Challenging, Cognitive Refocusing, Cognitive Reframing, Communication Skills, Exploration of Coping Patterns, Exploration of Emotions, Exploration of Nervous System Activation, Exploration of Relationship Patterns, Grounding/ resourcing, IFS, Interpersonal Resolutions, Intrapersonal Resolutions, Mindfulness Training, Open Ended Questioning, Practice Nonviolent Communication, Preventative Services, Psycho-Education, Structured Problem Solving, Supportive Reflection, Symptom Management, and Validation Exercises.

Treatment Plan Progress

Objectives

1. Client will learn and practice emotional regulation tools and techniques in session to use at least 3x a week outside of session to decrease the intensity, frequency, and consistency of client's "anxiety" and increase NS regulation. Client will decrease the distress connected to her "anxiety" levels from a level 8 to a level 6, over the next six months aeb client report in session.

Progress Note

Boulder Emotional Wellness

Clinician: Nicole Fortunato, LPC

Supervisor: Erica Johnson, LPC

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Progress: Progressing

2. Client will learn and implement somatic based exercises in session to decrease the somatic, emotional and mental responses connected to the client's "anxiety." Client will decrease the emotional, physical, and mental responses from a level 8 to a level 6, over the next six months aeb client report in session.

Progress: Progressing

3. Client will engage in active self-reflection and journaling at least 2x a week in moments after her NS has become activated and she is down regulated enough to track what happened before, during, or after NS response to bring more awareness to and learn the patterns at play and how her body responds to track connections between external and/ or internal stimulus and her NS response to decrease client's NS response time and activation period aeb client report in session.

Progress: Progressing

4. There are currently no obstacles to treatment regarding cultural, power, or other dynamics in the therapeutic relationship aeb client report in session.

Progress: Maintained

Assessment / Additional Notes

Clt. was at home for telehealth session.

Clt. has "consistent underlying anxiety" that increases when clt's. NS is triggered/ activated. clt. needs continued therapeutic support to continue decreasing clt's. anxiety and continuing to build and apply clt's. grounding and resourcing tools.

Plan

Th. and clt. plan to continue exploring clt's. anxiety and how it shows up for the clt. interpersonally and intrapersonally. Th. will hold space for clt. to share her experience, while offering grounding and resourcing tools, emotional validation, psych-ed, and somatic exercises to build clt's. capacity to be with her emotional experience and decrease her anxiety concurrently.

Recommendation: Continue current therapeutic focus

Prescribed Frequency of Treatment: Weekly

Nicole Fortunato, LPC, Licensed Professional Counselor, License CO 20828, signed this note and declared this information to be accurate and complete on 2/10/2026 at 11:30 AM.

Erica Johnson, LPC, Licensed Professional Counselor, License 14267, signed this note and declared this information to be accurate and complete on 2/10/2026 at 8:30 PM.